

Session Objectives:

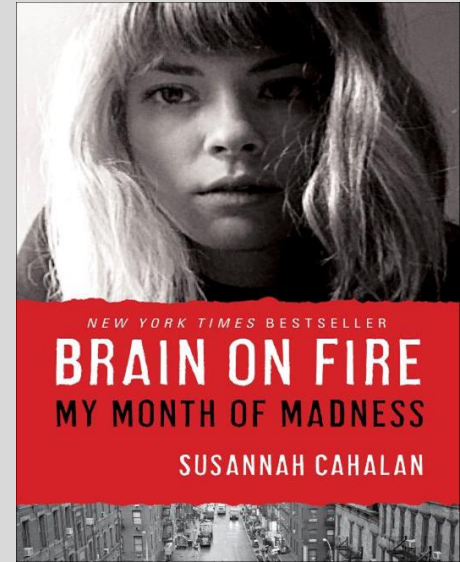
- 1) Distinguish neurological disorders from psychiatric disorders.
- 2) Recognize when psychiatric disease is comorbid with neurological disorders.
- 3) Identify when a patient is causing harm to him/herself.
- 4) Implement proper treatment for these conditions.

NMDAR encephalitis

- An autoimmune encephalitis that may be associated with ovarian teratoma or other rare tumor.
- Patients initially present with neuropsychiatric symptoms.
- Seizures may occur that are resistant to antiepileptic drugs (AEDs).
- May present with unusual movement disorders.

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Dopa responsive Dystonia

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- Also DYT 5 or Segawa disease
- Often presents with gait abnormality due to lower extremity dystonia. May be mistaken for cerebral palsy or spastic diplegia.
- Diurnal fluctuation: symptoms worsen in afternoon and improve in the morning after sleep.
- Treatment: Remarkable response to low dose levodopa.

Paroxysmal Kinesigenic Dyskinesia

- Dystonic posture or dyskinesia is brief- lasts seconds to minutes.
- Attacks triggered by sudden voluntary movements, stress, startle or sleep deprivation. May occur 100 times/day in some cases.
- Seizures may also be present.
- Responds to carbamazepine or other anti-seizure medications.

Split brain syndrome

- Callosal disconnection syndrome
- Although rare, this is the result of a patient who has had a lesion of the corpus callosum. May be due to:
- Surgery, MS, agenesis of corpus callosum
- Stroke, tumor, Marchiafava-Bignami disease
- Left hemisphere- calculations, reading and speech.
- Right hemisphere- spatial tasks

Transient Global Amnesia

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- Acute onset of anterograde amnesia (inability to form new memories).
- Repeatedly asks the same question reflecting disorientation.
- Other cognitive functions are normal.
- Eventually return to normal (generally within 24 hours).
- Presumed to either be migraine, ischemic, epileptic or psychogenic in origin.

Transient
Global
Amnesia



Factitious Disorder

- Occurs when an individual falsifies physical or psychological signs or symptoms or induces injury or disease with intended deception.
- The individual may create signs or symptoms by 1) lying 2)simulating-putting drops of blood into urine sample 3)actually creating disease by injecting bacteria.

Factitious Disorder

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- Primary aim is to achieve the patient role.
- Behavior persists even in the absence of monetary rewards or drugs, etc.
- The individual may seek invasive diagnostic testing, surgery or treatment.

Munchausen's Syndrome

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- This is the most severe variant of factitious physical disorder.
- Exaggerated lying AKA: pseudologia fantastica, sociopathic behavior, geographic wandering from hospital to hospital.
- Lives a continuous life of “patienthood”.
- The patient may also use aliases.

Body Dysmorphic Disorder

- BDD is characterized by a preoccupation with a perceived defect or flaw in the patient's appearance.
- Generally the flaw is not observed by others and the patient becomes obsessed with this particular flaw.
- The preoccupation triggers repetitive behaviors which are difficult to control and are not pleasurable.

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Pseudocyesis

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- The false belief that a patient is pregnant including growing abdomen, morning sickness and cessation of menstruation.
- Unclear if due to hormones or in some cases may be psychiatric.



Functional neurological Disorder

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- Neurologic symptoms such as weakness, abnormal movements, or nonepileptic seizures without structural disease.
- Clinical findings on exam are not compatible with recognized neurologic disease.
- The disorder causes distress and/or functional impairment and has a poor prognosis.

Malingering

- The patient consciously creates physical or psychological symptoms to assume the sick role with the goal of receiving a reward.
- The reward may include money, an insurance settlement, drugs, release from jail, avoidance from punishment, military, etc. May occur after a car accident (no fault).
- This is not a psychiatric illness and was not included in the DSM-V.

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Illness Anxiety Disorder

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- Excessive concern about having or developing a serious undiagnosed general medical disease. Previously patients were diagnosed with hypochondriasis in the DSM-4-TR.
- Patient has a belief that they have a specific disease. In DSM-5-TR



Case 17

- A 50-year-old woman was referred for evaluation of a movement disorder.
- When asked about her troublesome son she lies down, turns her head to the left and begins to shake violently.
- This continues for 5 full minutes. When it's over, she gets up and drives home.

Non-epileptic seizures (dissociative)

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- Characterized by sudden and time-limited disturbances of motor, sensory, autonomic, cognitive, and/or emotional functions that can mimic epileptic seizures.
- These events are **not** caused by abnormal brain activity (physical or mental stress).
- May occur in a patient who also has seizures.
- Treatment is psychotherapy.

Alice in Wonderland Syndrome

- A perceptual disorder involving brief, transient episodes of visual distortions.
- Can occur in conjunction with viral infections (EBV most common)
- Objects or body parts are perceived as altered in various ways.
- No permanent brain abnormalities are seen.

AIWS: Causes

- Other infectious causes (varicella, H1N1, coxsackievirus, typhoid fever, scarlet fever)
- Migraine
- Psychiatric disorders (major depression, schizophrenia)
- Epileptic seizures
- Medications (Topiramate, Risperidone, Montelukast, others)
- CNS lesions: infarct, tumor, hemorrhage
- Substance induced (cocaine, cannabis, MDMA, LSD others)

Phantom Limb Syndrome

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- Painful sensations referred to the missing limb (could also be breast, rectum)
- This condition was first described in the 16th century.
- Wars and land mines-responsible for many of the traumatic amputations over the years.
- Females and upper extremity amputation are at higher risk.
- Mirror therapy is used with a variety of other options.